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Title: *Current Ethiopian Healthcare System, HSTP*

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Acronyms

ART – Antiretroviral Therapy

BEmONC – Basic Emergency Obstetric and Newborn Care

CBHI – Community-Based Health Insurance

CEmONC – Comprehensive Emergency Obstetric and Newborn Care

CPD – Continuous Professional Development

DHIS2 – District Health Information Software 2

DP3 – Diphtheria, Pertussis, Tetanus (third dose vaccine)

eCHIS – electronic Community Health Information System

EFY – Ethiopian Fiscal Year

EHR – Electronic Health Record

GTP – Growth and Transformation Plan

GTP II – Growth and Transformation Plan II

HEP – Health Extension Program

HEWs – Health Extension Workers

HIS – Health Information System

HMIS – Health Management Information System

HPV – Human Papillomavirus

HSDP – Health Sector Development Programme

HSTP – Health Sector Transformation Plan

HSTP I – Health Sector Transformation Plan I

HSTP II – Health Sector Transformation Plan II

MCC – Motivated, Competent, and Compassionate (Health Workforce)

MoH – Ministry of Health

NCDs – Non-Communicable Diseases

NTDs – Neglected Tropical Diseases

OOP – Out-of-Pocket (expenses)

PHC – Primary Health Care

PMTCT – Prevention of Mother-To-Child Transmission (of HIV)

PPP – Public-Private Partnership

R&D – Research and Development

RHB – Regional Health Bureau

RMNCAH – Reproductive, Maternal, Neonatal, Child, and Adolescent Health

RMNCAH-N – RMNCAH + Nutrition

SDGs – Sustainable Development Goals

SHI – Social Health Insurance

UHC – Universal Health Coverage

Introduction

The Health Sector Transformation Plan (HSTP) is a comprehensive strategy aimed at improving the health status of the population by transforming the health sector. This plan also includes reforms and improvements in health education in order to build capable and competent health professionals and improve the quality of healthcare services.

Ethiopia's health system has undergone continuous reform since the early 1990s, responding to changing disease burdens, socioeconomic shifts, and international commitments. Ethiopia's health policy journey began in earnest with the 1993 National Health Policy, which prioritized primary health care, equity, and community participation. In alignment with the Alma-Ata Declaration (1978), the policy emphasized preventing and controlling communicable diseases and strengthening rural health access.

Over the next two decades, Ethiopia launched four consecutive Health Sector Development Programmes:

- **HSDP I (1997–2002):** Established district health offices, rolled out the National Health Account, and strengthened basic service packages.
- **HSDP II (2002–2005):** Expanded rural health extension workers, improved immunization, and reinforced essential drug supply systems.
- **HSDP III (2005–2010):** Integrated HIV/AIDS, tuberculosis, and malaria control into primary care; scaled up facility deliveries and antenatal services.
- **HSDP IV (2010–2015):** Focused on quality improvement, health management information systems, and performance-based financing.

Each phase built on lessons learned, progressively widening service coverage, enhancing human resources, and introducing accountability mechanisms across all regions.

The Health Sector Transformation Plan (HSTP I) was the next five-year national health sector strategic plan, which covered EFY 2008-2012 (July 2015 – June 2020). It was prepared by conducting in-depth situational assessment and performance evaluation of HSDPs; considering the global situation and the country's global commitment and most importantly, the goals of the national long-term vision and Growth and Transformation Plan (GTP). Ethiopia has accomplished the second growth and transformation plan (GTP-II, June 2015 – June 2020) marked by several positive results and achievements.

Then comes the second Health Sector Transformation Plan (HSTP-II), which was the next five-year national health sector strategic plan. It covers the period between

2013–2017 Ethiopian fiscal years (July 2020–June 2025). During this strategic period, the sector envisions building on the successes and consolidating the gains of HSTP-I to build a resilient, sustainable, high-quality, equity-based health system. Thus, the preparation of HSTP-II was informed by in-depth situational analysis of the performance of the health sector during HSTP-I. The plan also took into account the nation’s long-term socioeconomic strategic directions and priorities, the global situation and country’s commitments to sustainable development goals, and the dynamics of social determinants of health.

1. Health Sector Transformation Plan I (HSTP I)

I. Introduction

In 2015, the Federal Ministry of Health of Ethiopia launched the Health Sector Transformation Plan I (HSTP I) as a five-year road-map (2015/16–2019/20) to guide the country's health sector reform. The overarching vision of HSTP I was to create a healthy, productive, and prosperous society by achieving Universal Health Coverage (UHC), where every citizen would have access to quality, equitable, and affordable health services without financial hardship.

The development of HSTP I was not in isolation but was strategically aligned with Ethiopia's Growth and Transformation Plan II (GTP II). This alignment ensured that health sector interventions contributed to the broader national development agenda. HSTP I emerged at a time when Ethiopia had made measurable gains in health outcomes but still faced deep-rooted challenges, particularly in service accessibility, quality, and health financing.

II. Strategic Focus Areas

HSTP I was built around five major strategic pillars, each addressing a critical dimension of Ethiopia's health system.

A. Strengthening Primary Health Care (PHC)

PHC was identified as the cornerstone of Ethiopia's health system. The government invested heavily in upgrading health posts, health centers, and district hospitals. The Health Extension Program (HEP) was expanded to ensure coverage in rural and hard-to-reach areas. Key initiatives included:

- Construction and upgrading of over 3,000 health centers and 300+ district hospitals.
- Deployment of more than 38,000 Health Extension Workers (HEWs).
- Enhanced supply chains for essential medicines and commodities.
- Increased focus on preventive and promotive health services.

B. Reproductive, Maternal, Neonatal, Child, Adolescent and Youth Health (RMNCAH)

Improving RMNCAH indicators remained a priority, with focused interventions to reduce maternal and neonatal mortality:

- Promotion of institutional deliveries through maternity waiting homes.
- Expanded immunization campaigns (e.g., EPI and new vaccines like HPV).
- Integrated adolescent and youth health programs in schools and health facilities.
- Scale-up of family planning access through public and community-based distribution.

C. Health Workforce Development

Human resources were at the center of service delivery reforms. Major milestones included:

- ❖ Expansion of health professional training institutions across regions.

- ❖ Implementation of continuous professional development (CPD) programs.
- ❖ Introduction of performance-based incentives in selected woredas.
- ❖ Strategic placement of health workers in underserved areas using bonding agreements and hardship allowances.

D. Health Financing Reform

To reduce financial barriers and ensure long-term sustainability, HSTP I introduced several reforms:

- Community-Based Health Insurance (CBHI) rolled out in over 700 woredas, covering millions of informal sector workers.
- Expansion of Social Health Insurance (SHI) schemes for formal sector employees (though progress remained slow).
- Introduction of health facility revenue retention and autonomy mechanisms.
- Resource mobilization from domestic and donor sources was prioritized, but significant gaps remained.

E. Health Information System (HIS) Strengthening

Reliable data became a core enabler for planning and monitoring:

- ❖ The Health Management Information System (HMIS) was digitized and streamlined.
- ❖ Introduction of the District Health Information Software (DHIS2) to standardize reporting.
- ❖ Use of real-time dashboards for data-driven decision-making at the woreda and regional levels.
- ❖ Emphasis on improving data quality, completeness, and timely reporting.

III. Achievements and Results

HSTP I was successful in driving substantial improvements in health outcomes, access and systems performance. These gains were measured through key health indicators and coverage milestones.

A. Progress in Health Outcomes

- Maternal Mortality Ratio (MMR) declined from 420 (per 100,000 live births in 2014) to 401 by 2019, although the target of 267 was not fully achieved.
- Under-5 mortality decreased from 64 to 55 deaths per 1,000 live births, and infant mortality dropped from 44 to 39.
- Life expectancy increased from 64 years in 2014 to 66.6 years by 2019, owing to improved primary care access and disease control programs.

B. Service Coverage Gains

Skilled Birth Attendance rose from 28% to over 50% during HSTP I.

DPT3 immunization coverage improved to 94%, and measles coverage exceeded 90%.

Increased access to tuberculosis treatment, HIV services, and malaria prevention (e.g., insecticide-treated nets, IRS).

Facility delivery rates increased, thanks to community mobilization and transportation vouchers in remote areas.

C. Alignment with Sustainable Development Goals (SDGs)

HSTP I contributed significantly to SDG 3: “Ensure healthy lives and promote well-being for all at all ages.” Notable contributions included:

Reductions in maternal and child mortality.

Improved access to sexual and reproductive health services.

Stronger emphasis on health equity, particularly for pastoralist and hard-to-reach populations.

IV. Challenges

1. Geographic Disparities in Health Service Access:

A major barrier encountered was the unequal distribution of health services between urban and rural areas. While cities and regional capitals benefited from expanded infrastructure and human resources, many remote, rural and pastoralist communities continued to face limited access. Geographic isolation, poor transportation infrastructure, and under-resourced facilities created a gap in equitable service delivery.

2. Frequent Stock-Outs and Weak Supply Chains:

Health facilities in several regions faced recurring shortages of essential medicines, laboratory supplies, and equipment. These interruptions in the supply chain not only hindered service quality but also eroded public trust. The health system's logistic and inventory management systems were not consistently reliable especially at the woreda and health center levels.

3. Gaps in Health Financing and High Out-of-Pocket Costs:

Although Ethiopia made strides by introducing Community-Based Health Insurance (CBHI), the scale and depth of coverage remained limited. Many citizens still bore significant out-of-pocket expenses when seeking care, leading to delays in treatment or complete avoidance of the health system due to cost. Budget limitations at federal and regional levels also restricted the scale-up of planned reforms.

4. Health Workforce Shortages and Maldistribution:

While the government increased the number of training institutions and graduates, retaining skilled professionals—especially in hard-to-reach areas—remained a challenge. Health workers were often overburdened, inadequately supervised, and lacked opportunities for continued professional growth. This had implications for both morale and the quality of care.

5. Weaknesses in Health Information and Data Use:

The Health Management Information System (HMIS) expanded under HSTP I, but gaps in data quality, completeness, and timely reporting persisted. Health workers in many facilities lacked the training and tools needed to collect and use data effectively. As a result, planning and decision-making at district and facility levels were often based on incomplete evidence.

V. Lessons Learned:

1. Community Engagement Yields Results:

The Health Extension Program (HEP) proved to be a cornerstone in improving rural health. Communities that were actively engaged—through health development armies and outreach programs—showed better uptake of services like antenatal care, immunization, and family planning. Ownership and participation at the local level increased both demand and accountability.

2. Health Requires Multisectoral Action:

The experience of HSTP I reinforced that health outcomes are influenced by more than just clinical services. Improvements in sanitation, nutrition, education, water access, and infrastructure were shown to have direct and indirect effects on health. Effective collaboration across government sectors was key to holistic development.

3. Technology Alone Is Not Enough:

While digital tools for health data collection and service delivery were introduced, their impact was limited without proper training and institutional support. Capacity-building in digital literacy and data interpretation must accompany any technological rollout to be successful.

4. Flexible, Data-Informed Planning Enhances Resilience:

During implementation, the need for adaptive policymaking became clear. Health programs that were able to use real-time data to respond to disease outbreaks, low coverage indicators, or shifting population needs were more effective. Resilience was tied to a system's ability to learn and adapt.

5. Sustained Investment in Workforce Retention Matters:

Training more health professionals is important—but it is equally essential to retain them through improved working conditions, supportive supervision, career advancement, and incentive structures. A stable and motivated workforce is at the core of any strong health system.

2. Health System Transformation Plan II (HSTP II)

HSTP-II (2020 – 2025; 2013-2017) follows HSTP-I (2015 -2020), during which significant improvements in Maternal Mortality, Under 5 Mortality, life expectancy and infant mortality were seen.

Vision

To see a healthy, productive, and prosperous society

Mission

To promote the health and well-being of the society through providing and regulating a comprehensive package of health services of the highest possible quality in an equitable manner.

Values

- Community first, Integrity, loyalty, honesty

- Transparency, accountability, and confidentiality
- Impartiality,
- Respect for law
- Being a role model
- Collaboration
- Professionalism
- Change/innovation and Compassion

I. Objectives

The objective of HSTP-II is strengthening the health system to ensure that people live longer, healthier lives, expanding access to high-quality health care for all and improving the effects of social determinants of health. It helps to achieve the goal of never leaving anyone behind. The objective of HSTP-II is to improve the health status of the population by realizing four main objectives:



1. Accelerate progress towards universal health coverage

This objective focuses on accelerating the progress towards full coverage of essential health services and protecting people from financial hardship. This objective ensures the achievement of the following three components of UHC to all population subgroups:

- ✓ Essential service availability: to ensure that all components of care are available at each service delivery level, with an acceptable level of quality.
- ✓ Essential service coverage: to ensure that all individuals and communities receive the services they need. By combining 3 essential components: need, utilization and quality.
- ✓ Financial risk protection: access to all needed quality health services without being exposed to financial hardship

2. Protect people from health emergencies

This objective focuses on improving health security by protecting the public from the impact of public and medical health emergencies caused by human-made and natural disasters and new health threats. It also includes safeguarding the public from cross-border health problems and ensuring the health security of the population.

3. Woreda transformation

This objective focuses on an aspiration to see a transformed Ethiopia at each Woreda, a pathway towards development by using households as building blocks of nations and means to cascade SDG to sub national level so that no one is left behind by tailoring/customizing national programs to local context and creating ample space for local wisdom and ownership.

4. Improve health system responsiveness

Health system responsiveness refers to the level to which health services respond to the needs and expectations of targeted individuals and communities. This objective focused on respecting and responding to preferences and values of individuals and communities during health service provision, ensuring that beneficiaries' values dictate health service delivery processes.

II. Targets

Health Sector Transformation Plan II (HSTP II: 2020/21–2024/25) include 76 detailed targets across its strategic pillars.

Below is a structured breakdown of how these 76 targets are organized.

1. By Strategic Pillar

The 76 targets are distributed across four pillars of HSTP II:

A. Quality and Equity in Health Services (24+ targets)

- Maternal Health:
 - Increase skilled birth attendance to 90% (from 73% in 2019).
 - Reduce obstetric fistula cases by 50%.
- Child Health:
 - Ensure 95% of under-5 pneumonia/diarrhea cases receive treatment.
 - Reduce stunting to 25% (from 38% in 2019).
- Equity:
 - Increase health service coverage in pastoralist regions to 75% (e.g., Somali, Afar).

B. Disease Prevention and Control (20+ targets)

- HIV/TB/Malaria:
 - Achieve 95-95-95 HIV targets (diagnosis, treatment, viral suppression).
 - Eliminate malaria in 50% of districts by 2025.
- NCDs:
 - Screen 50% of adults for hypertension/diabetes annually.
 - Reduce salt intake by 30% (for cardiovascular disease prevention).

C. Health System Strengthening (18+ targets)

- Workforce:
 - Train 10,000 new midwives and 5,000 surgeons by 2025.
 - Ensure 80% of hospitals meet staff-to-patient ratios.
- Infrastructure:
 - Upgrade 100% of primary hospitals to provide emergency surgery.
 - Achieve 24/7 electricity and water in 90% of health facilities.

D. Digital Health and Governance (14+ targets)

- Technology
 - Roll out electronic health records (EHRs) in all referral hospitals.
 - Implement telemedicine in 50% of zones.
- Accountability:
 - Conduct annual public health expenditure reviews.
 - Reduce corruption in procurement by 30%.

2. Cross-Cutting Targets

- Nutrition: Reduce anemia in women of reproductive age to 20% (from 24%).
- Climate Resilience: Ensure 50% of health facilities have disaster preparedness plans.
- Community Engagement: Train 50,000 community health volunteers for outreach.

3. Data Sources for Tracking

- HMIS (Health Management Information System): Tracks service coverage (e.g., antenatal visits, vaccinations).
- Surveys: Demographic and Health Surveys (DHS) and WHO reports validate progress.

III. Strategic directions

Strategic directions for a policy are the high-level, overarching goals and priorities that guide the development and implementation of a specific policy. They define the desired future state the policy aims to achieve and provide a framework for making decisions about resource allocation and specific actions. Essentially, strategic directions answer the question: What are we trying to accomplish with this policy, and what are the most important things to focus on?

The strategic plan for HSTP-II identifies 14 strategic directions that are detailed below.

- 1. Enhance provision of equitable and quality comprehensive health services:** focuses on provision of health promotion, disease prevention, curative, rehabilitative, and palliative care services in an equitable manner and at the highest possible quality.
- 2. Improve health emergency and disaster risk management:** focuses on public health emergency and disaster management and the elements of this process are effective and timely anticipation, prevention, early detection, rapid response, control, recovery, and mitigation of any public health emergency crises.
- 3. Ensure community engagement and ownership:** focuses on ensuring active participation and engagement of the community in planning, implementation, monitoring and evaluation of health and health related activities.
- 4. Improve access to pharmaceuticals and medical devices and their rational and proper use:** focuses on strengthening the pharmaceutical supply chain, pharmacy

services, and medical device management systems to ensure uninterrupted availability and accessibility of safe, effective, and affordable medicines and medical devices.

5. Improve regulatory systems: focuses on protection of the public from health risks that arise from poor and substandard products and services.

6. Improve human resource development and management: focuses on improving the quality of pre-service training and continuous professional development. It also emphasizes promotion of ethics and professionalism in pre-service education and in-service training programs.

7. Enhance informed decision making and innovations: focuses on building a culture of evidence-based decision-making, and developing and using technology

8. Improve health financing: focuses on ensuring adequate and sustainable financing to achieve Universal Health Coverage without financial hardship for citizens.

9. Strengthen governance and leadership: focuses on ensuring an accountable, transparent leadership and governance system.

10. Improve health infrastructure: focuses on improving access to health facilities that are well equipped and furnished, and ensures that all health institutions meet minimum standards.

11. Enhance digital health technology: This strategic direction includes four major components: Digitization targeted to clients, health workers, health system managers, and health data services.

12. Improve traditional medicine: focuses on registration, licensing, research, production, use, and integration of traditional medicine and traditional medical practices.

13. Ensure integration of health in all policies and strategies: focuses on considering the health implications of decisions of public policies across all sectors.

14. Enhance private engagement in the health sector: focuses on improving the engagement of the private sector in improving access and quality of health services, and to increase their engagement in a comprehensive range of health-related activities

IV. Priorities /Transformation Agendas of HSTP-II

HSTP-II of Ethiopia focuses on addressing key challenges in the health system by identifying five core transformation agendas (priority areas). These transformation agendas are designed to address systemic issues and build a more equitable, effective, and resilient health system in Ethiopia under HSTP-II. The five priorities of HSTP-II are:

1. Quality and Equity

Goal: Ensure reliable, patient-centered, efficient, and timely care is available to everyone—regardless of age, gender, socioeconomic status, location, or disability.

Major Actions:

- Improve both demand and supply for quality care.

- Raise public expectations to encourage demand for quality.
- Establish accountability and transparency in service delivery.
- Monitor equity regularly and implement tailored interventions (e.g., new service delivery models, incentive schemes).

2. Information Revolution

Goal: Build a culture and system that values and uses high-quality data for decision-making and improved health system performance.

Three Pillars:

- Culture of data use.
- Digitization of the Health Information System (HIS).
- Improved HIS governance.

3. Motivated, Competent, and Compassionate (MCC) Health workforce

Goal: Ensure an adequate, well-trained, and emotionally invested health workforce to deliver quality care.

Key Interventions:

- Strengthen pre-service and in-service education, and CPD (Continuous Professional Development).
- Ensure fair recruitment, placement, and performance support.
- Provide equitable remuneration, clear job roles, and enabling work environments.
- Strengthen HR management policies and practices.

4. Health Financing

Goal: Reform health financing to ensure sustainable funding and efficient resource use.

- Challenges Addressed:
- High out-of-pocket costs and financial barriers to care.
- Inefficiencies in resource allocation.

Major Interventions:

- Mobilize more resources (domestic & international).
- Reform resource allocation and prioritization.
- Strengthen health insurance systems.
- Encourage public-private partnerships.
- Implement performance-based financing and cost recovery reforms.

5. Leadership and Governance

Goal: Enhance leadership, accountability, and governance at all levels of the health system.

Key Issues:

- Lack of transparency, coordination, regulation, and shared vision.

Key Interventions:

- Restructure the health system.
- Institutionalize accountability.
- Strengthen clinical governance and regulatory autonomy.

- Improve stakeholder engagement and partnerships.
- Build leadership capacity.
- Mainstream the Health in All Policies approach across sectors.

V. HSTP-II: Key Programmatic Areas (2020/21–2024/25)

1. Reproductive, Maternal, Neonatal, Child, and Adolescent Health and Nutrition (RMNCAH-N)

Why it's a priority:

- ✧ High maternal and child mortality rates persist.
- ✧ Ethiopia's performance in RMNCAH is critical for achieving SDGs 3.1 (maternal health) and 3.2 (child health).

Strategic Interventions:

- ✧ Improve access to **antenatal, delivery, and postnatal care**
- ✧ Expand **skilled birth attendance and Basic/Comprehensive Emergency Obstetric Care (BEmONC/CEmONC)**
- ✧ Promote **family planning**, targeting both demand and supply
- ✧ **Scale up adolescent and youth-friendly health services**
- ✧ Improve **nutrition services**, including IYCF and micronutrient supplementation

Targets:

- ✧ Reduce **MMR to 140 per 100,000** live births
- ✧ Reduce **under-5 mortality to 140 per 1,000** live births
- ✧ Increase **modern contraceptive prevalence rate (mCPR)**

2. Communicable Diseases Prevention and Control

Why it's a priority:

- Ethiopia still faces a high burden of HIV, TB, malaria, and NTDs.

Strategic Interventions:

- ✧ **HIV/AIDS:** Improve case detection, ART adherence, and PMTCT
- ✧ **Tuberculosis:** Expand GeneXpert testing, TB preventive therapy (TPT), and community TB care
- ✧ **Malaria:** Intensify insecticide-treated net (ITN) coverage, indoor residual spraying (IRS), and early diagnosis
- ✧ **NTDs:** Implement mass drug administration and hygiene promotion for diseases like trachoma, schistosomiasis

Goals:

- ✧ Achieve **epidemic control of HIV**
- ✧ Reduce **TB incidence and mortality** by 50% from baseline
- ✧ Eliminate **malaria** in selected low-transmission areas

3. Non-Communicable Diseases (NCDs), Mental Health, and Injuries

Why it's a priority:

- NCDs are rising due to urbanization, lifestyle changes, and aging population.
- Mental health and injuries (road traffic accidents, violence) are emerging burdens.

Strategic Interventions:

- ✧ Integrate **NCD screening and management** (hypertension, diabetes, cancers) at PHC level
- ✧ Strengthen **mental health** services within general hospitals and health centers
- ✧ Establish systems for **injury prevention and trauma care**

Key targets:

- ✧ Increase NCD service coverage
- ✧ Reduce **premature mortality** from NCDs
- ✧ Improve availability of **essential medicines** for NCDs

4. Emergency, Critical, and Surgical Care

Why it's a priority:

- Critical care and emergency systems are weak and unequally distributed.

Strategic Interventions:

- ✧ Establish **emergency medical teams and referral systems**
- ✧ Strengthen **trauma and surgical services** at district and regional levels
- ✧ Introduce and scale **ambulance services and emergency preparedness plans**

Targets:

- ✧ Functional **emergency units** in all primary hospitals
- ✧ Availability of **basic surgical care** in general hospitals

5. Environmental and Occupational Health

Why it's a priority:

- Poor water, sanitation, air quality, and occupational hazards contribute to disease burden.

Strategic Interventions:

- ✧ Ensure **access to safe water, sanitation, and hygiene (WASH)**
- ✧ Enforce **occupational safety standards** in workplaces
- ✧ Address **air pollution, food hygiene, and waste management**

Goals:

- ✧ Increase households with access to **basic sanitation**
- ✧ Improve **school and health facility WASH coverage**

6. Health Promotion and Disease Prevention

Why it's a priority:

- Prevention is more cost-effective than treatment and essential for UHC.

Strategic Interventions:

- ✧ Promote **behavior change communication (BCC)** and **community engagement**
- ✧ Use **health extension program (HEP)** to educate households
- ✧ Implement **school health programs, youth clubs, and media campaigns**

Targets:

- ✧ Increase knowledge and uptake of **healthy behaviors**
- ✧ Strengthen **community ownership** of health outcomes

7. Cross-Cutting Issues

These are integrated across all program areas to ensure inclusive and resilient health systems.

Cross-Cutting Area	Key Focus
Gender equality & women's empowerment	Mainstream gender in all services
Disability inclusion	Provide equitable services to persons with disability
Urban health	Address unique challenges of urban poor
Migration and refugees	Provide services in border and conflict areas
Climate change & health	Build climate-resilient health facilities
Digital health	Use eHealth and mHealth to improve service delivery and data management

VI. Strengths of HSTP-II

The strength of a Health Sector Transformation plan II (HSTP-II) lies in its ability to address the key challenges facing the healthcare system and to implement effective solutions for improving health outcomes. Some of the key strengths of the health sector transformation plan include:

1. Commitment toward UHC

Universal health coverage (UHC), is about attaining effective coverage of essential health services and protecting people from financial hardship. HSTP-II targets to Increase UHC index from 0.43 to 0.58 and according to World Health Statistics Data 2019 (Comparable estimates) there is Improvement over baseline and achieved more than 70% to 85% of the MTR targets. This was achieved by:

- Expanded Health Service Coverage: Substantial increase in access to essential health services including maternal and child health, immunization, and infectious disease control. Improvement in service availability in rural areas via health extension programs and PHC (Primary Health Care) expansion.
- With respect to financial risk protection there is Implementation and expansion of Community-Based Health Insurance (CBHI) and pilot Social Health Insurance (SHI) schemes which helped to reduce out-of-pocket (OOP) expenditure for basic services in some regions.
- Improved Health indicators like reduction in maternal, infant, and under-five mortality, Improvement in immunization coverage and reduced communicable diseases (e.g., malaria, TB, HIV).
- Digital Health Technology: This system has facilitated the registration of approximately 80 digital health systems, ensuring proper documentation and

source code submission Telemedicine and electronic community health information systems (eCHIS) introduced.

2. Alignment with National and Global Frameworks:

HSTP II is aligned with Ethiopia's 10-year development plans, Sustainable Development Goals (SDGs), and global health commitments, ensuring coherence with broad socio-economic priorities.

3. Emphasis on Quality and Equity

HSTP II clearly prioritizes quality improvement and narrowing health disparities between geographic, gender and socioeconomic groups.

4. Enhance public-private partnership

HSTP II facilitate the private sector's engagement in the expansion of health infrastructure, local production of pharmaceuticals, and medical devices, as well as training and continuing development for health professionals. It also encourages foreign investment in tertiary and specialized health services, and development of pharmaceutical industries with incentives.

5. Promotes decentralized health system

HSTP II promotes decentralization in Ethiopia's health system primarily through Woreda Transformation as a Core Objective, Integrated Primary Health Care and Expanded Services at Local Levels, Strengthening Governance and Leadership at Sub-National Levels, Data-Driven Decision Making and Information Revolution throughout the System, Health Financing and Community-Based Health Insurance (CBHI) and Multi-sectoral and Community Engagement.

VII. Limitations and challenges of HSTP-II

Although it had its own strengths over the previous policies. It isn't without limitations. Some of the limitations and challenges for HSTP II includes:

1. The share of general government expenditure on health as a share of general total government expenditure is not progressing well. It is still below 9% and quite low compared to the target set. Looking at the share of the health budget in the total government budget at different levels of administration sheds light on the extent to which the challenge exists. In this regard, the share of total health budget to total government budget at the federal level is small (max. 6.6% in 2013 EFY) compared to regions allocation of 10-15% of their total budget, with the exception of Addis Ababa (7%). In addition to the small share of the health budget in total government expenditure at the federal level, even this small share has decreased over the last three years (6.6%, 5.9%, and 3.4% in 2013, 2014, and 2015 EFY, respectively).
2. Low health worker density and inequitable distribution of health workers are also critical areas to be addressed. The national health worker density varies greatly from region to region and from rural to urban areas. Inadequate health workforce motivation, retention, and performance management mechanisms

are still a concern due to a lack of budget and uniform motivation and incentive packages.

3. Health In All Policies and multi-sector coordination require effort and commitment from all sectors, but not all sectors contribute equally and there is a gap in follow up by line Ministries. Multi sectoral engagement lack regularity and structure. There is no guideline for implementation of health in all policy in Ethiopia. And as a result, there is budget limitation, limited awareness and knowledge about the health in all policies and limited gender mainstreaming for multisector activities.
4. Lack of prior experience and limited awareness on transactional PPP made it difficult to convince decision makers and get potential bidders from the private sector. Decision making is a time consuming process
5. Currently the efforts made to improve traditional medicine are more fragmented as there are too many stakeholders acting separately such as Health, Education, Agriculture, Environment, Industry, Culture & heritage, and others to exploit the rich source and untapped knowledge of traditional medicine in the country. There is limited protection and preservation of indigenous knowledge resulting in lack of trust among the traditional healers and the researcher's impeding collaboration for validation of traditional remedies. There is also limited interest and support for traditional medicines specifically for R&D, training of professionals, practitioners and the community.
6. Absence of Health information system structure in some Regional Health Bureau's. Health construction projects financed through regional budgets are executed by Construction Bureaus. Construction Bureaus have no specific department that follows up health projects. In addition, they lack experts with a specialty in managing health infrastructure which asks for unique expertise by way of familiarity with HF standards, knowing the service flow, types of equipment, etc. Hence, they face challenges in terms of meeting standards, considering the workflow, progress. In addition, there is weak information flow to MOH as there is no reporting line between construction Bureau and the MOH.
7. The systems are challenged with inadequate health IT human resource capacity in terms of the skill mix, numbers, and skill sets of health IT professionals needed to support the implementation and sustainability of digital health systems. There is also weak device management and tracking system, including maintenance capacity. The major reason for all the shortfall has been availability of limited of financial resources to scale up digital systems as per the plan. This lack of finances has resulted in a wide disparity in digital health implementation. Many of the digital health initiatives are taking place in Addis Ababa and other main cities of the country. There is a visible inequity in HIS and

Digital health systems implementation. Limited multisectoral engagement outside of Ethio-Telecom has also been reported as another challenge.

8. General Health professionals and health and health related institutions regulatory bodies lack independency and legal framework to operate on a legal ground .The Regulatory is organized as a Lead Executive Office under the MOH. Similarly, regional regulatory structures are quite diverse and most lack indolence (they are Directorates under the RHB) and are not well budgeted and staffed. The regulator lacks an adequate number of staff and the required professional mix such as physicians; pharmacists etc. are in dire shortage. It also faces shortage of budget and support from partners across the board (federal as well as in regions).
9. Inequity in Access and Quality: Disparities between urban vs. rural, rich vs. poor, regions (e.g., emerging regions lag behind). Gaps in quality and respectful care practices.
10. Infrastructure and Supply Chain Issues: Inadequate infrastructure (e.g., narrow space, poor ventilation, limited equipment). Frequent stockouts of essential drugs and diagnostics.

3. Lessons for designing HSTP- III

- ✓ Invest in the design and implementation of ‘catch-up’ initiatives to rebound from service disruptions during the COVID-19 pandemic and other shocks (conflict, climate-related threats such as drought) since the start of HSTP II implementation.
- ✓ It's not only about reaching everyone, but reaching them with quality care so focus must shift from service expansion to improving the quality and equity of services across all regions.
- ✓ The health workforce is the backbone of the health system. HSTP-III must invest in training, retention, motivation, and equitable distribution of skilled health workers, especially in hard-to-reach areas.
- ✓ Strengthening decentralization is essential for equitable performance. Regional performance was inconsistent due to weak capacity and support. HSTP-III must provide stronger leadership, technical support, and accountability mechanisms to regional health bureaus.
- ✓ Revisit the design health service delivery architecture by setting clear, measurable service norm/standards (e.g., infrastructure, financing, HR) for each level of care and modality (e.g., static site, outreach, mobile health services, home visits/home-based care, telehealth) and develop PHC investment plan, thus HSTP-III must further invest on;
 - a) rational infrastructure strengthening (rehabilitation, renovation, new construction)-including HEP optimization;
 - b) increased service availability (including slowly evolving services such as adolescent health, NCDs, mental health) in existing facilities

c) introduction of new service delivery modalities
 d) Optimize service integration and efficiencies in service provision
 Strengthening referral services across the continuum of care nationally and at local levels

- ✓ Invest further in public-private partnerships to extend health access
- ✓ HSTP-III must further improve financial risk protection by enhancing health financing reforms—scale up CBHI, Social Health Insurance, and reduce reliance on donor funding.
- ✓ Integrate Non-communicable Diseases into primary care and develop national programs for screening, treatment, and health promotion.
- ✓ Design and implement ‘catch-up’ initiatives and innovative service delivery platforms (e.g., mobile service delivery, telehealth) to address existing inequities in service delivery, service disruptions and backlogs.
- ✓ Health is shaped by more than healthcare—it needs multi-sector partnerships like water, sanitation ,education, infrastructure, etc. So formalize and strengthen multi-sector coordination and partnerships

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